Designing for Negotiation in Collaborative Healthcare, Interactive Health 2026: Statement of Interest

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1 Introduction

Healthcare is rarely shaped by a single form of knowledge. In practice, decisions about care, and about the tools that support it, emerge through ongoing negotiation across clinical expertise, institutional priorities, and the lived realities of the people most affected. Our work is driven by a question that sits at the heart of this workshop: how might AI systems be designed to surface, mediate, and leverage these multiple forms of knowledge?

We are researchers at Monash University's Action Lab, working in Human-Computer Interaction and digital health. We want to express our interest in participating in this workshop to develop our understanding of, and contribute our own insights to, a meaningful discussion on the role of AI in healthcare. In particular, we want to explore how AI can be leveraged to design more equitable and collaborative healthcare spaces that better support diverse stakeholders and communities.

2 Our Work and Context

Ananya Alagh is a PhD student and Research Fellow at Monash University's Action Lab. Her work spans community-based rehabilitation and maternal health, focusing on contexts where care is distributed across multiple actors and where collaboration is actively negotiated across differences in knowledge, authority, and lived experience. In a project with community-based disability workers in rural India, she designed and evaluated a virtual community of practice to support peer learning and knowledge exchange, examining how institutional structures and existing hierarchies shape what counts as legitimate knowledge in care. More recently, her work has focused on maternal and perinatal health for communities in Australia, designing digital interventions to support access to timely and relevant health information through participatory co-design with community members and healthcare providers. Drawing on a systems-oriented approach to understanding her work examines how knowledge flows and authority shape the conditions under which

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decisions are made in care, and examines ways in which to incorporate lived experience at crucial points within care pathways.

Ling Wu is a Senior Research Fellow in digital mental health and human-centred design at Monash University. Her work is grounded in a concern with how design methods can broaden what is recognised as valid evidence in intervention development. For her, human-centred and participatory design are not simply approaches for improving usability or acceptability; they are epistemic practices that bring lived experience into the conversation about what should count as evidence in shaping a health intervention. She has worked across a range of digital health contexts, including maternal health, parenting, and mental wellbeing, and is especially interested in collaborative health contexts in which design decisions are negotiated across multiple stakeholders, including patients, families, clinicians, service organisations, and researchers, and in how AI might mediate the interpretation, weighting, and uptake of lived experience evidence in shaping digital intervention outcomes.

3 Our Perspectives and Motivation

Our motivation to participate in this workshop emerges from a shared concern that runs across our projects; that AI has a role to play in mediating collaborative healthcare interactions, as well as in the very processes through which health interventions and digital systems are designed. We are interested in how AI might support the use of lived experience: surfacing, weighing, factoring in the knowledge and realities that communities bring to shaping healthcare systems. Specifically, we want to deepen our understanding of how AI might redistribute or broker power between patients, caregivers, clinicians, and researchers in the delivery of care, and extend this question into the design process itself, examining how AI might help incorporate lived experience evidence into shaping the systems and tools we build.

This concern is grounded in a persistent tension we have each encountered in practice. Participatory and co-design approaches are increasingly common in digital health, yet the final design decisions often remain largely expert-driven. Lived experience findings are more readily incorporated when they align with existing professional assumptions, intervention logics, or institutional expectations. When they challenge those assumptions, their implications might be softened, reframed, or excluded from the final design outcome. We see AI as a potential mediator in this process, one that could support more equitable brokering of power between the forms of expertise that shape digital health systems, between patients and clinicians in the delivery of care, as well as between researchers, institutions, and communities in the design process itself.

We are exploring these ideas through *BabyReady*, a maternal health initiative that investigates how an AI-driven platform can reshape the delivery of health information to better align with users' needs, concerns, and contexts. Rather than presenting standardised guidance, the system is designed to generate tailored, narrative-driven content that responds to what users want to know, how they want to engage, and what feels relevant to them. This includes content shaped around personal circumstances, family and cultural contexts, and everyday constraints, so that the information users receive is not generic health guidance but information that speaks to their situation. In doing so, BabyReady moves beyond passive information provision: by giving users content shaped around their own realities, it redistributes some of the power that has traditionally sat with healthcare providers, enabling users to engage with the health system from a more informed position. The system becomes a mechanism for power brokering, allowing users to engage with healthcare material in a way that aligns with their realities and interests, and affords them a greater deal of autonomy over their healthcare journey.

Through the design and ongoing development of *BabyReady*, we are generating insights into how these dynamics unfold, particularly in contexts where trust, representation, and access are already uneven. This includes understanding how users respond to personalised content, how they interpret system-generated information, and what supports or constrains their ability to engage with it critically. This work does, however, raise questions that extend beyond what a single project can address. We are curious, for instance, about how others have navigated the tension between personalisation and objectivity in AI-driven health content, and about what design strategies have proven meaningful in contexts where trust in both the technology and the healthcare system itself is fragile. We are also keen to understand how other researchers and practitioners are thinking about the role of AI in the design process, not only as a tool for delivery, but as a potential means of keeping lived experience in view as interventions are shaped and refined over time.

We see this workshop as an opportunity to bring our own ideas and insights into dialogue with others working across healthcare settings, and to collectively advance our understanding of how AI-mediated systems can support negotiation, shared understanding, and more situated and equitable forms of decision-making in care.